

7 Study Parameters

TREATMENT MUST BEGIN ≤ 7 WORKING DAYS FROM RANDOMIZATION

Procedure	Induction Phase (fulvestrant + everolimus or placebo for a maximum of 12 cycles)					End of Induction Phase <OR> End of Treatment Section 5.6^	Continuation Phase [£] (until progression or unacceptable toxicity)	Follow-Up Off Therapy ^o (every 3 months for total of 3 yrs from date of randomization)
	Pre-Study	Cycle 1 Day 1*	Cycle 1 Day 15	Day 1 Each Subsequent Cycle*	Every 12 weeks			
Eligibility Assessment	X ^a							
Informed Consent	X							
Medical History & Assessment of Baseline Signs and Symptoms	X ^a							
Physical Exam	X ^a	X ^b	X	X ^c		X		
Concomitant Meds (see Appendix C)	X ^a	X ^b	X	X ^c		X		
Vital Signs (Blood Pressure, Heart Rate, Temperature) and Weight	X ^a (includes Ht)	X ^b	X	X ^c		X		
ECOG Performance Status	X ^a	X ^b	X	X ^c		X		
Adverse Events Assessments		X ^b	X	X ^c		X ^j	X ^j	X ^j
Imaging Scans	X ^{a,i}				X ⁱ	X ⁱ	X ⁱ	X ^{i,m,o}
PT with INR	X ^a							
Urinalysis	X ^a							
CBC with differentials, Platelets	X ^a	X ^b	X ^d	X ^{c,d}		X ^d	X ^k	
Serum Creatinine, Electrolytes (K, Na, Cl, CO ₂), Ca, BUN, Albumin, Total Protein, Phosphorus, AST (SGOT), ALT (SGPT), Alkaline Phosphatase, Total Bilirubin, Magnesium, Uric Acid.	X ^a	X ^b	X	X ^c		X	X ^k	
Fasting Glucose	X ^a	X ^b		X ^e	X ^e	X ^e	X ^k	
Fasting serum Lipid Profile (triglycerides, total cholesterol, HDL and LDL)	X ^a	X ^b		X ^e	X ^e	X ^e	X ^k	

Procedure	Induction Phase (fulvestrant + everolimus or placebo for a maximum of 12 cycles)					End of Induction Phase <OR> End of Treatment Section 5.6^	Continuation Phase [£] (until progression or unacceptable toxicity)	Follow-Up Off Therapy [°] (every 3 months for total of 3 yrs from date of randomization)
	Pre-Study	Cycle 1 Day 1*	Cycle 1 Day 15	Day 1 Each Subsequent Cycle*	Every 12 weeks			
HBV-DNA, HbsAg, HBsAb, HBcAb, HCV-RNA-PCR	X ^{a,f}							
HBV DNA		X ^{f,g}		X ^{f,g}			X ^k	
HCV RNA-PCR		X ^{f,h}		X ^{f,h}			X ^k	
Study Drug Compliance (Pill Diary)			X	X		X	X ^l	
PFTs with DLCO as medically indicated ⁿ								

a: ≤ 4 weeks of randomization; if assessments required ≤ 7 days of Cycle 1 Day 1 (C1D1), they do not need to be repeated (includes labs).

b: ≤ 7 days prior to the start of C1D1.

c: +/- 72 hour window allowed prior to D1 of each subsequent cycle after the first cycle for scheduled therapy/tests/visits. Delay due to holidays, weekends, bad weather or other unforeseen circumstances will be permitted.

d: In the event of grade 3 or 4 hematologic toxicity, CBC with differential and platelet count will be obtained every 1-3 days until there is evidence of hematologic recovery.

e: +/- 72 hours prior to Cycle 2 Day 1 then approximately every 12 weeks during treatment (and more frequently as clinically indicated), and at end of treatment.

f: All patients should be screened for hepatitis risk factors and any past illnesses of hepatitis B and hepatitis C infection (see Section 7.1.1). All patients with a positive medical history per Section 7.1.1 need hepatitis testing as noted on above table. It is highly recommended that patients positive for HBV-DNA or HBsAg are treated prophylactically with an antiviral (e.g., Lamivudine) for 1-2 weeks prior to receiving study drug (see Table 5-3). The antiviral treatment should continue throughout the entire study period and for at least 4 weeks after the last dose of everolimus.

g: Patients on antiviral prophylaxis treatment or positive HBV antibodies should be tested for HBV-DNA ≤ 7 days prior to the start of C1D1 and +/- 72 hrs prior to D1 of each subsequent cycle to monitor for reactivation. See Table 5-4 for reactivation instructions.

h: Patients with positive HCV RNA-PCR results at screening and/or a history of past infection (even if treated and considered 'cured') should have HCV RNA-PCR testing performed on ≤ 7 days prior to the start of C1D1 and +/- 72 hrs prior to D1 of each subsequent cycle to monitor for flare. Everolimus must be discontinued if HCV flare is confirmed according to the guidance in Table 5-5.

i: Tumor measurements may be made using physical examination, CT Scans or MRI. Tumor assessments will be performed every 12 weeks, +/- 1 week (every 3 months). Imaging will include chest and abdomen. Bone Scans and Brain CT/MRI may be performed as clinically indicated. Scans do not have to be repeated once disease progression is documented.

j: Adverse events related to fulvestrant and/or everolimus/placebo will be followed for 30 days after the last dose of study therapy (fulvestrant and/or everolimus/placebo) or until ≤ grade 1 or if the grade is >1, the event must be permanent and stable. Please note- Serious adverse events >30 days after last dose of fulvestrant and/or everolimus/placebo are not reported unless the event may be related to everolimus/placebo.

k: CBC and chemistry may be used to assess ongoing toxicity but are not required in the Continuation Phase for patients who receive fulvestrant alone. Patients who continue fulvestrant with everolimus should periodically have CBC, chemistries, fasting glucose, fasting lipids, HBV DNA, HCV RNA-PCR per labeling guidelines.

l: Study Drug Compliance (Pill Diary) for those patients who receive everolimus in the Continuation Phase.

m: All patients including those that discontinue protocol therapy will be followed for 3 years from the time of randomization. Patients that have not progressed during the Induction or Continuation Phase will continue to have imaging scans completed every 12 weeks, +/- 1 week (every 3 months) until documented progression.

n: PFTs with DLCO as medically indicated only, (PFTs are not otherwise required during course of study).

o: Follow every 3 months for disease progression and survival. Initiation of any new systemic therapy will also be documented.

* Cycle 1, Day 1 is defined as the first day on which fulvestrant is given in combination with placebo/everolimus (the second fulvestrant dose is given on day 15 of the first cycle only). Day 1 of each additional cycle is defined as the day in which fulvestrant is given in combination with everolimus/placebo.

^ End of Induction/End of Treatment should be performed within 30 days of last dose of fulvestrant.

£ Continuation Phase: Patients in the Continuation Phase should continue to receive fulvestrant alone (if originally randomized to placebo) or in combination with everolimus (if originally randomized to everolimus) at the same dose and schedule (+/- 1 week window for scheduled therapy/tests/visits; delays due to holidays, weekends, bad weather or other unforeseen circumstances will be permitted) until disease progression or unacceptable toxicity.